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CLK/jw

Clinic date: 01.03.17

02 March 2017

Dr S. Samuel
The Hillview Surgery
114 Finchley Lane
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Dear Dr Samuel

Mr Moshe SHAMASH – dob: 13.12.1942
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Thank you for referring this very pleasant 75-year-old left-handed retired gentleman who attended with his son and daughter. For the past two months he has been aware of tenderness over his scalp (he points to the area between the left parietal region and the vertex). He says it is not painful. He is not aware of any discomfort if he doesn't touch it. Sometimes when he turns over in bed at night he is aware of the symptoms. He does not have similar symptoms in his face. He has never suffered from headaches. He is not aware of any precipitating factors.

He suffers on and off from gout. He had a benign cyst removed from a kidney 15 years ago. At the age of 12 he had a traumatic event when a nail entered his right eye and he has lost the vision in the right eye and wears a false cover over the eye.

He does suffer from hypertension but no diabetes, asthma or raised cholesterol. Blood tests performed in October 2016 and again two days ago. The results are awaited.

He has been under quite a bit of stress for the past two years. He has no allergies. He is a passive smoker in that his wife and friends smoke. He does not drink alcohol.

His father lived to the age of 96. His mother died in a road traffic accident. He is one of 11 siblings. His eldest brother recently passed away at the age of 86 and nine years previously he had a stroke. A sister died at the age of 83. One sister suffered from leukaemia but an older brother gave her a bone marrow transplant and she is now well.

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The patient is currently taking *Enalapril* 10mg daily, *Bendroflumethiazide* 2.5mg daily, *Lipitor* 10mg daily and *Diltiazem Hydrochloride SR* 120mg daily.

On examination, he looked well. His pulse rate was 68 bpm regular and blood pressure 120/70. He was blind in the right eye. There was no papilloedema on examination of the left fundus. Full eye movements were present in the left eye. There was no tenderness or induration of the superficial temporal arteries. Normal pulsation was palpable over these arteries. There was no tenderness over the temporomandibular joints. No tenderness on touching the scalp and sensation was intact.

There was no meningism or tenderness of the cervical muscles.

Motor and sensory examinations were normal. The reflexes were present and equal and the plantar responses were flexor. Coordination testing was normal. Gait was normal, tandem walking was normal and Romberg's sign was negative.

I concluded that he presents with some sensitivity over his scalp on the left side.

I have referred him for blood tests, particularly to check his activity markers (ESR and CRP). I have also requested a MRI scan of the brain.

I will phone him as soon as I receive the result of the scan as I believe he is travelling overseas this weekend.

I will review him on 27th March 2017. He does not feel he requires any medication at the moment.

With best wishes,

Yours sincerely

(Electronically approved)

Dr Charles Kaplan
Consultant Neurologist

CC: Mr Moshe Shamash
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